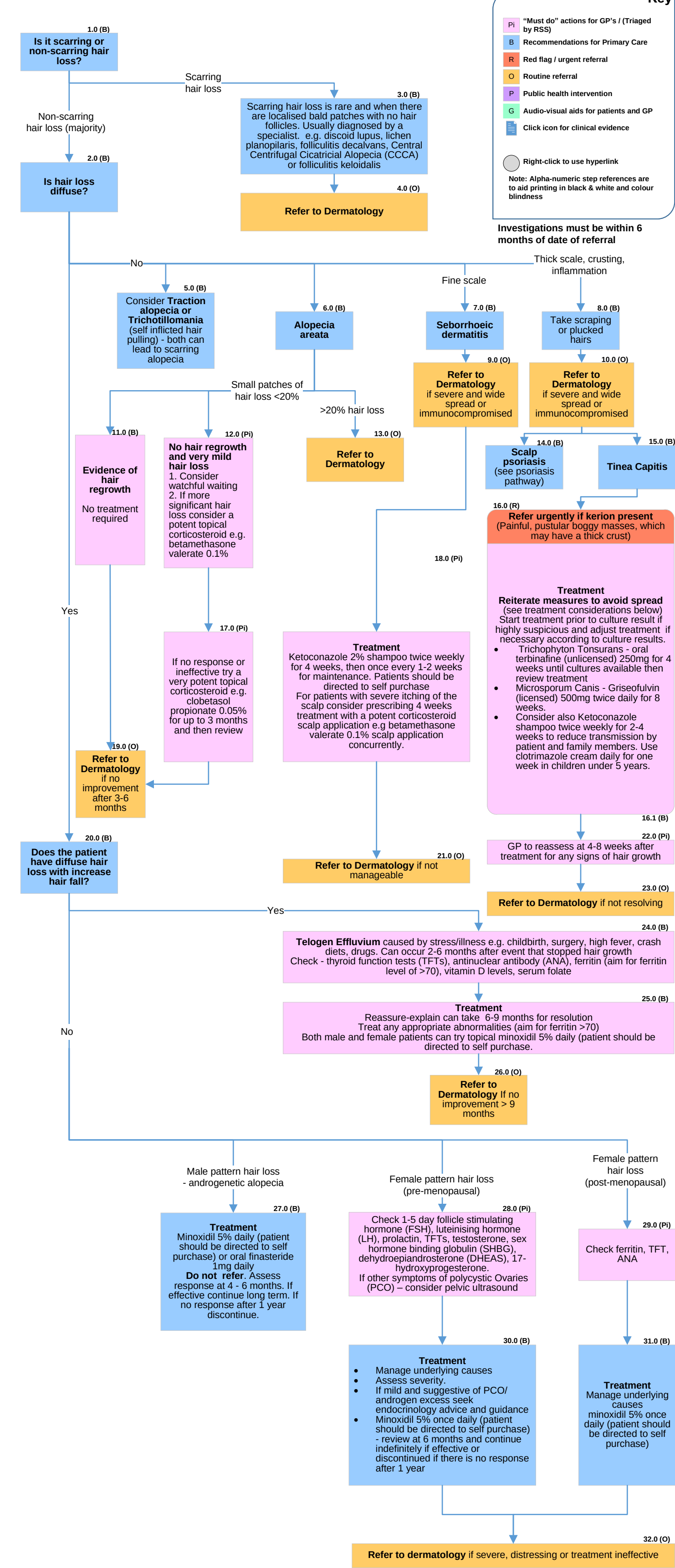


North Central London (NCL) ICS

Hair Loss (adult) Primary Care Clinical Pathway

For any queries regarding the content of this document e-mail: nclcb.pathways@nhs.net

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Treatment/Drug considerations	
- Potent or very potent topical corticosteroids should not be used on beard area or eyebrows - Selenium sulphide shampoo is an alternative to ketoconazole shampoo for seborrheic dermatitis. Only available over the counter. Use twice weekly for 2 weeks. Selenium sulphide shampoo contraindicated in pregnancy - Seborrheic dermatitis - mild crust or scale can be removed with warm mineral or olive oil. A keratolytic preparation e.g. salicylic acid and coconut oil maybe needed for thicker scale, then washed out with a suitable product eg coal tar shampoo - Tinea capitis/prevention of spread - avoid sharing towels, combs, helmets etc. Disinfect or discard objects that can transmit fungal spores e.g. hats, pillows, hairbrush etc. - Griseofulvin and terbinafine should be avoided in pregnancy - Griseofulvin may reduce effectiveness of combined contraceptive pill	
Please refer to the Summary of Product Characteristics (SPC) of any drug considered. This pathway has been developed from published guidance in collaboration with local dermatologists. This guidance is to assist GPs in decision making and is not intended to replace clinical judgement	
References https://cks.nice.org.uk/fungal-skin-infection-scalp https://cks.nice.org.uk/seborrheic-dermatitis#scenario:1 https://cks.nice.org.uk/alopecia-androgenetic-female https://cks.nice.org.uk/alopecia-androgenetic-male https://cks.nice.org.uk/alopecia-areata http://www.pcids.org.uk	

TABLE 1: IMAGES OF CONDITIONS ASSOCIATED TO THE NCL ADULT HAIR LOSS PRIMARY CARE CLINICAL PATHWAY		
CONDITION	IMAGE	DESCRIPTION / NOTES
Scarring hair loss		Scarring alopecia Discoid lupus erythematosus (DLE) is the most common chronic form of cutaneous lupus. It is characterised by persistent scaly, disk-like plaques on the scalp, face, and ears that may cause pigmentary changes, scarring and hair loss. https://www.skinhealthinfo.org.uk/condition/discoid-lupus-erythematosus-dle/ Lichen planopilaris typically presents as smooth white patches of scalp hair loss. No hair follicle openings can be seen in the areas of hair loss. Scale and redness surround each hair follicle at the edges of these patches, which may be spiny on palpation. Hairs can be easily pulled out. It is multifocal and small areas may merge to form larger irregular areas. More common in young women than men. There are several forms including follicular lichen planus, frontal fibrosing alopecia and Graham Little syndrome. https://www.skinhealthinfo.org.uk/condition/lichen-planopilaris/ Folliculitis decalvans is a rare chronic inflammatory condition of the scalp. Very rarely it can affect other hair-bearing skin such as the beard, armpits, pubic area and legs. Prolonged inflammation leads to scarring. Folliculitis decalvans is not contagious and is not cancerous. https://www.skinhealthinfo.org.uk/condition/folliculitis-decalvans/ Central Centrifugal Cicatricial Alopecia (CCCA) is a form of scarring alopecia on the scalp that results in permanent hair loss. It is the most common form of scarring hair loss seen in black women but may be seen in men and persons of other race or hair colour. Hair loss typically begins at the vertex or mid-scalp and extends outward in a centrifugal manner. There is loss of the follicular openings on the scalp. Thus, the scalp may appear shiny. Symptoms of tenderness, itch and burning are common. Hair loss is slowly progressive. https://www.skinhealthinfo.org.uk/condition/central-centrifugal-cicatricial-alopecia/
Scalp psoriasis		May occur in isolation or with any other form of psoriasis. The back of the head is a common site, but multiple discrete areas of the scalp or the whole scalp may be affected. Scalp psoriasis is characterised by thick scale over well-defined, red thickened skin. The scale is often silvery white. Psoriasis may extend slightly beyond the hairline. May be asymptomatic but can be extremely itchy. Can fluctuate in severity and extent. In very severe cases, there may be some temporary mild localised hair loss, but scalp psoriasis does not cause permanent balding. https://www.skinhealthinfo.org.uk/condition/psoriasis/
Seborrheic dermatitis		Very common. Seborrheic dermatitis can affect almost half of adults. Slightly commoner in men https://www.skinhealthinfo.org.uk/condition/seborrheic-dermatitis/
Tinea Capitis		Ring-shaped, scaly, red rash, most common in children. Caused by dermatophytes. Contagious through close contact with an infected person, or by sharing combs, hairbrushes, hats, clothing, towels, beds and other furniture. Can be caught from infected animals. The fungus can live for long periods of time in the environment and therefore infection can occur many months later. Symptoms include itching, redness and dryness of the scalp. Sometimes bald patches can occur. Pustules, yellow crusts and matted hair and, very rarely, a kerion. In these cases, patients may have a fever or swollen, painful glands in the neck. https://www.skinhealthinfo.org.uk/condition/tinea-capitis/ Patient leaflet: https://www.bad.org.uk/shared/get-file.ashx?id=1965&itemtype=document
Traction alopecia or Trichotillomania		Hair loss caused by constant pulling, often due to pulling repeatedly by tight hairstyles. Trichotillomania is patient actively pulling their own hair. Itching, soreness or increased flaking of the scalp can occur. https://www.skinhealthinfo.org.uk/condition/traction-alopecia/
Alopecia Areata		Non-scarring, occurs at any age. Usually small, coin sized round patches of baldness on the scalp. The beard, eyebrows, eyelashes and limbs can also be affected. If eyelashes affected, eye irritation may occur. https://www.skinhealthinfo.org.uk/condition/alopecia-areata/ Patient leaflet: https://www.bad.org.uk/shared/get-file.ashx?id=68&itemtype=document
Telogen Effluvium		Usually affects whole scalp. Occurs suddenly and up to 3 months after trigger such as childbirth, severe trauma/illness, marked weight loss, extreme dieting etc. https://www.skinhealthinfo.org.uk/condition/telogen-effluvium/

CONDITION	IMAGE	DESCRIPTION / NOTES
Kerion		Presents as a boggy pus-filled lump, often several centimetres in diameter. It is characterised by marked inflammation. Hairs within the kerion are loose and fall out, often resulting in a bald area. Enlargement of the regional lymph nodes can occur, and some people become systemically unwell with fevers and malaise. It may be followed by a widespread itchy eczema-like rash
Male pattern hair loss - androgenetic alopecia		Androgenetic alopecia - male pattern hair loss Occurs in 50% of men over 50 years. Usually no physical symptoms other than hair loss which is the front/top of the head or receding hairline. https://www.skinhealthinfo.org.uk/condition/male-pattern-hair-loss-androgenetic-alopecia/ Patient leaflet: https://www.bad.org.uk/shared/get-file.ashx?id=3831&itemtype=document
Female pattern hair loss		Androgenetic alopecia – female pattern hair loss Severity can vary. Can be associated with PCOS. Onset usually later than males. Usually widespread thinning of the hair mainly on the crown of the scalp, front hairline remains normal. https://www.skinhealthinfo.org.uk/condition/female-pattern-hair-loss-androgenetic-alopecia/ Patient leaflet: https://www.bad.org.uk/shared/get-file.ashx?id=3830&itemtype=document
Folliculitis keloidalis		Unusual form of chronic folliculitis and cicatricial alopecia at the nape of the neck. More common in dark skinned people. Most common in Afro-Caribbean males with black curly hair. Twenty times more common in males than females. Initially itchy, round, small bumps appear around the occipital scalp. Pustules may be present and scratching may lead to secondary bacterial infection. Bumps can become small scars which enlarge to become keloid masses. https://dermnetnz.org/topics/folliculitis-keloidalis

With thanks to Dermnetz for images